National Cancer Control Programme (NCCP) 00456 · clinician draft

Cisplatin + etoposide + chest radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Cisplatin + etoposide

Active medicines: Cisplatin + Etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on the platinum day;80 mg on the next2 days	Relative to each cisplatin dose
Ondansetron	16 mg orally	Before the cisplatin dose Adjust for QT/individual risk.
Dexamethasone	12 mg orally on the platinum day;8 mg daily on the next3 days	Reference high-risk course Reconcile interactions and any other steroids.
Olanzapine	5 mg orally on the platinum day and next3 days	Reference high-risk course Sedation, metabolic risk and other medicines require review.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row has no end-day orPRN qualifier. Confirm duration; coadministration with olanzapine increases extrapyramidal risk.

Additional phase-specific support

- Multiday systemic anticancer therapy (SACT): reconcile antiemetics for each day's medicines and the two days after; one combined prescription is required. Do not add independent overlapping courses.
- The5-day cisplatin aprepitant extension is not automatically applicable: this regimen gives cisplatin only on Days 1 and 8.
- Coordinate radiation-related nausea, swallowing, nutrition, skin care and respiratory assessment with radiation oncology.
- No automatic routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is specified in these NCCP regimens; consider individual risk and local policy.
- Review etoposide interactions, live vaccines, nephrotoxic/ototoxic medicines and any duplicate QT-prolonging antiemetics.
- Cisplatin pre/post hydration and electrolytes per local policy. NCCP suggests posthydration10 mmol magnesium sulphate plus20 mmol potassium chloride in 1 L sodium chloride0.9% over2 hours; adjust to patient and local policy.
- Day 1 etoposide in 1 L may be considered prehydration; Day 8 has no etoposide infusion, so prescribe its own prehydration. Routine furosemide to induce urine flow is not recommended without fluid overload.
- NCCP reduced count rows still require reconciliation with the etoposide SmPC. Count rows apply to etoposide and do not clear cisplatin independently.
- After grade 3 non-haematological toxicity (excluding mucositis/alopecia), wait for recovery to grade 1 or less; the source 75% restart requires dose-history and denominator review. Grade 4 is outside that source row.
- After grade ≥2 neuropathy recovers to ≤1, a consultant chooses carboplatin AUC5 substitution or 50% cisplatin. Do not restore full-dose cisplatin automatically or substitute the separate AUC3 regimen.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00456 v4 · pNCCP 00456 supportive care; NCIS V8 Table 1 and multiday section · NCCP NCIS antiemetics V8

Sources checked 2026-11-07 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required